

FAX TRANSMITTAL COVER SHEET
PRIVILEGED AND CONFIDENTIAL INFORMATION

LONG ISLAND NEUROSURGICAL ASSOCIATES P.C.
&
COHEN CHILDREN'S MEDICAL CENTER PEDIATRIC NEUROSURGERY

410 LAKEVILLE ROAD, STE #204
NEW HYDE PARK, NY 11042
TEL# (516) 354-3401
FAX# (516) 354-8597
WWW.LINEUROSURGERY.COM

- Steven J Schneider, MD ()
- Mark A. Mittler, M.D ()
- Michael A. Lefkowitz, MD ()
- Shaun D. Rodgers, M.D ()
- Shanna Baron, FNP ()
- Yael Horowitz, FNP ()
- Cathy Grimm - Admin ()
- Staff Member ()

Urgent

DATE: 11/28/23

TO THE ATTENTION OF: Medical review

FAX NUMBER: 846-889-8061

THIS TRANSMISSION HAS 11 PAGES INCLUDING COVER SHEET

FROM: [REDACTED]

REGARDING: [REDACTED]

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Procedure Order

Ordering Site

Long Island Neurosurgical Main Office
410 Lakeville Rd.suite 204
New Hyde Park, NY 11042
(516) 354-3401
Fax: (516) 354-8597

Report Date: 05/17/2023

Patient Information

[REDACTED]
[REDACTED]
[REDACTED]

Patient Insurance Information

[REDACTED]
[REDACTED]

Collection Agency
Group #NONE
Plan #NONE

Procedures Ordered

MRI - Brain W/O Contrast (70551) *Case # 1198919129*
Diagnosis: ARACHNOID CYST OF SPINE (G96.198) Ordered by: Shanna L. Baron, FNP

MRI CERVICAL SPINE W/O CONTRAST (72141) *Case # 1198920203*
Note: PLEASE PROVIDE PATIENT WITH DISC
Diagnosis: ARACHNOID CYST OF SPINE (G96.198) Ordered by: Shanna L. Baron, FNP

MRI THORACIC SPINE W/O CONTRAST (72146) *Case # 1198920939*
Note: PLEASE PROVIDE PATIENT WITH DISC
Diagnosis: ARACHNOID CYST OF SPINE (G96.198) Ordered by: Shanna L. Baron, FNP

MRI LUMBAR SPINE W/O CONTRAST (72148) *Case # 1198921629*
Note: PLEASE PROVIDE PATIENT WITH DISC
Diagnosis: ARACHNOID CYST OF SPINE (G96.198) Ordered by: Shanna L. Baron, FNP

End of Procedures Ordered

05/08/2023

05/08/2023

05/08/2023

05/08/2023

Date

Shanna Baron FNP-C

WWW.LINEUROSURGERY.COM

F:

P002/011

Nov 28 2023 11:03am

Fax: 516-354-8597

L I NEUROSURGICAL

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Patient Message

mom called

To: [REDACTED]
From: [REDACTED]
Sent: 5/8/2023 2:42 PM
Phone: (631) 424-6304

mom called said that [REDACTED] have been having headaches and has a weird "bubble" feeling in his head. Do you want to send him for a scan first or come in for eval. Please advise, I can call mom back to let her know # mom [REDACTED]

11/28/2023 10:59 am

Veradigm EHR 23.4
Allscripts

Page 1/1

11/18/2020 01:19 PM

Weight: 163 lb Height: 70 in

Body Surface Area: 1.91 m² Body Mass Index: 23.39 kg/m²

Temp.: 96.6 °F

Assessment & Plan

ARACHNOID CYST OF SPINE (G96.198)

■ returns to office for follow-up visit. He has occasional interscapular pain. The recent MRI from 11/2/2020 reveals that the ventral epidural arachnoid cyst has decreased in size. It previously spanned from C3-L4, and now spans from T8-9 to L4. I am pleased with his progress.

Plan:

Follow-up with physical therapy and possibly pain management for muscle spasm symptoms.

Follow-up as needed.



Steven J. Schneider MD

NORTHWELL HEALTH
LONG ISLAND JEWISH MEDICAL CENTER

270-03 7605 AVENUE, NEW HYDE PARK, NEW YORK, 11040 • (516) 470-7000

PATIENT NAME: [REDACTED]
DATE OF OPERATION: 07/21/2017
MEDICAL RECORD #: [REDACTED]
ENCOUNTER #: [REDACTED]
SURGEON: [REDACTED]
DATE OF BIRTH: [REDACTED]

OPERATIVE REPORT

ASSISTANT: [REDACTED]

PREOPERATIVE DIAGNOSIS: Extradural and intradural cystic spinal lesion.

OPERATION: L2 and L3 left hemilaminectomy, L2-L3 medial facetectomy and foraminotomy for combined extradural and intradural cystic lesion with midline approach, reconstruction of dorsal elements, intraoperative ultrasound, and enhanced surgical surfaces for difficult anatomic location of cystic lesion

POSTOPERATIVE DIAGNOSIS: Extradural and intradural cystic spinal lesion.

ANESTHESIA: General endotracheal intubation.

ESTIMATED BLOOD LOSS: [REDACTED]

INDICATIONS: This is a young man with a history of carnitine palmitoyl transferase deficiency and a large intramedullary lesion. It appeared to be primarily extradural with a possible intradural extension after two myelograms. It is extremely complex, and the patient is symptomatic with pain. The risks, benefits, and potential complications of exploring this lesion, intradural and extradural were explained and accepted, including, but not limited to, infection, bleeding, anesthesia, permanent and temporary neurologic deficits, residual recurrent lesion, CSF leak, the need for additional procedure including a shunt, and even death. Aseptic meningitis and inflammatory reaction were also discussed. It was to explore this lesion which was between the pedicles of L2 and L3 with a hemilaminectomy and medial facetectomy and intraoperative visualization, possible intradural exploration. It was all explained and accepted in great detail.

PROCEDURE: The patient was brought to the operating room and placed under general anesthetic. The spinous process of L3 marked and methylene blue inserted on a sterile prep. X-ray confirmation obtained, and the area was prepped and draped in the usual fashion.

A midline incision was carried over the spinous processes of L2 and L3. Subperiosteal dissection was performed on the left side to expose the lamina and the spinous processes on the left side of L2 and L3 as well as the pars interarticularis at that level. Midas Rex was used, and a curette was used to then enter the epidural space and separate the ligamentum flavum

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PATIENT NAME: [REDACTED]
MEDICAL RECORD #: [REDACTED]
DATE OF BIRTH: [REDACTED]

from the undersurface of the L3, and hemilaminectomy performed with the Midline of the 2 and 3 levels. It was cut from the medial facet as well. Kerrison rongeur was used and then extended the medial facetectomy and foraminotomy. Care was taken to preserve the interarticularis. The L3 nerve root and L2 nerve root were well visualized. Intraoperative monitoring carried out which was benign. We were able to stimulate the root at quadriceps contraction consistent with the L3 root, and then we carefully began dissecting the epidural vasculature to enter the epidural space.

We at that time did not visualize any significant intradural or extradural communication. We found the cystic lesion anteriorly. It was a difficult dissection but eventually we were able to visualize it coming to the level of L3. At this point we brought in the operating microscope as we looked for an intradural communication. It was unclear as giant cysts could occupy the region. These needed to be coagulated and sectioned. With the ultrasound we should see the lesion. We opened the dura and directly visualized the intradural area. We did not see any obvious intradural communication. We carefully explored from L2 to L4 laterally all the way past the midline. Ensuring that we extended the section from the L2 to L4 nerve roots and their pedicles. We fenestrated and drained the inferior portion of the cystic lesion. We did not see it feeding from the dura despite having seen probable communication on myelography. At this point, we had no obvious CSF leak. The cyst had drained nicely and decompressed. At this time, we irrigated out copiously with irrigation and closed with running Gore-Tex in a tight fashion. A small amount of fat was harvested from the incision and was placed into the epidural space and glued into position with DuraSeal and Surgagen placed over the area.

At this time, we went ahead and meticulously hemostases. We reconstructed the dorsal elements in a laminaeplasty technique at L2 and L3 utilizing PDS sutures through the ligamentum flavum. We closed the muscle and fascia with running absorbable interrupted PDS. Subcutaneous tissue closed with interrupted PDS, and then skin with monocrystalline. At this point, he tolerated the procedure well. Monitoring was stable, and he was taken to the recovery room in a stable condition moving his extremities well.

Please note that the operating surgeon for the beginning of the case to the end of the case. It was a difficult procedure. We utilized the operative microscope for the majority of the dissection intradurally and extradurally for the cystic lesion.

DICT: STEVEN J SCHNEIDER, M.D. (07123) 07/21/2017 03:53 PM
TRANS: V_TSRAA_TV_TSPRV_Q 07/21/2017 08:58 PM
JOB: 2182908

11/03/2020 3:55:58 PM -0500

NORTHWELL HEALTH

Northwell Reichert Family Imaging at Huntington

An Extension of North Shore University Hospital
234 Plaisant Road, Great Neck, NY 11740 (833) 670-8458 (631) 670-5497

Department of Radiology

MR#: [REDACTED] EPI #: [REDACTED]
DOB: [REDACTED] AGE: [REDACTED]
Acct #: [REDACTED]
Home#: [REDACTED]
Order #: 90030



EXAM: MR SPINE CERVICAL

EXAM: MR SPINE LUMBAR

EXAM: MR SPINE THORACIC

PROCEDURE DATE: 11/02/2020

INTERPRETATION: CLINICAL INFORMATION: ROUTINE . ADM/DIAG1: G96.198 OTHER DISORDERS OF MENINGES, NOT ELSEWHERE CLASSIFIED/. Arachnoid cyst status post fenestration. Follow-up.

TECHNIQUE: Multiplanar multisequence noncontrast MRI of the cervical, thoracic and lumbar spine was performed.

COMPARISON: MRI cervical, thoracic and lumbar spine 5/14/2019.

FINDINGS:

There is a ventral epidural lesion extending from the T8-T9 level to the L4 level. The lesion demonstrates fluid signal and internal septations, suggesting an arachnoid cyst. This is decreased in size compared to 5/14/2019, when it extended from C3 through L4. There is mass effect on the thecal sac. No cord signal abnormality.

Status post left L2 and L3 hemilaminotomies. Vertebral body heights and alignment are within normal limits. Marrow signal is preserved. Intervertebral disc signal is preserved. No significant neuroforaminal narrowing.

05/17/2019 4:59:16 PM -0400

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An Extension of North Shore University Hospital
284 Pulaski Road, Greenvale, NY 11740 (631) 670-3456 (631) 670-3497

Department of Radiology

ADDENDUM

Patient: [REDACTED]
AKA: [REDACTED]
MR#: [REDACTED] EPI #: [REDACTED]
DOB: [REDACTED] AGE: [REDACTED]
Acct #: [REDACTED]
Home #: [REDACTED]
Order #: 90028



EXAM: MR SPINE CERVICAL

EXAM: MR SPINE THORACIC

EXAM: MR SPINE LUMBAR

*** ADDENDUM 05/17/2019 ***

History: Multilevel arachnoid cyst follow-up MRI.

*** END OF ADDENDUM 05/17/2019 ***

PROCEDURE DATE: 05/14/2019

INTERPRETATION: Examination: MRI of the cervical, thoracic and lumbar spine.

TECHNIQUE: Sagittal T1-weighted, T2-weighted, and inversion recovery images, and axial T1 and T2-weighted images of the cervical, thoracic and lumbar spine were obtained.

Comparison: MRI of the cervical, thoracic and lumbar spine dated 7/25/2018, 11/20/2017

FINDINGS:

No significant interval changes regarding the size or appearance of the extensive ventral epidural multiseptated CSF collection extending from C3 all the way through L4. The collection abuts the thecal

LEWIS WAKIDJIAN, D.O., ATTENDING RADIOLOGIST

As the Attending Physician, I personally performed the professional interpretation of the study, personally supervised the professional interpretation of the study and/or reviewed the images and the interpretation of the resident, and have edited the report where appropriate. If I do not personally perform the procedure, I personally performed or was present and supervised the critical or key portions of the procedure, and have written or reviewed and edited the report where appropriate.

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